Depression (Major Depressive Disorder): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Major depressive disorder (MDD) is a mood disorder. It involves a lasting change from a person's usual self, marked by **persistently low mood and/or a loss of interest or pleasure**, along with changes in body, thinking, and behavior. It is episodic: a person can have one episode, but recurrence is common. It causes real distress or impairment in work, school, relationships, or self-care. It is not ordinary sadness, and it is not normal grief, although grief can turn into depression. It is also not explained better by a substance, a medical condition, or a bipolar-spectrum disorder.

Counselor framing: depression is common, treatable, and often recurrent. Assessment always includes screening for bipolar history (any past mania or hypomania), substance use, medical contributors (for example, thyroid disease or medication effects), and safety.

2. Core symptoms to watch for

The DSM-5-TR criteria are summarized here in plain language. Use the manual itself for formal diagnosis.

- **Duration:** symptoms are present most of the day, nearly every day, for **at least two weeks**.
- **Count:** at least **five** symptoms from the list below. **At least one** of the five must be low mood or loss of interest/pleasure.
    1. Depressed, sad, empty, or hopeless mood. In children and teens this may show up as irritability.
    2. Clearly reduced interest or pleasure in most activities (anhedonia).
    3. Significant change in appetite or weight, up or down.
    4. Sleeping too little or too much.
    5. Visible slowing down, or visible restlessness and agitation, that others can observe.
    6. Fatigue or loss of energy.
    7. Feelings of worthlessness or excessive or inappropriate guilt.
    8. Trouble concentrating, thinking, or making decisions.
    9. Recurrent thoughts of death, suicidal thoughts, a plan, or an attempt.
- **Related presentations:** *Persistent depressive disorder* means chronic depressed mood lasting **two years or more** in adults (one year in youth). Specifiers such as anxious distress, melancholic features, psychotic features, peripartum onset, and seasonal pattern shape treatment planning.

Suicide risk note (general clinical consensus): ask directly about suicidal thoughts at intake and throughout treatment. Asking does not plant the idea. Assess ideation, plan, intent, access to means, past attempts, and protective factors. Use a structured tool (for example, the C-SSRS, or PHQ-9 item 9 as a screen), develop a collaborative safety plan, counsel on reducing access to lethal means, and consult or refer when risk is elevated. Watch especially for hopelessness, recent losses, agitation, substance use, and periods of transition (for example, early improvement in energy, or discharge).

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in depression. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
PHQ-9 (Patient Health Questionnaire-9)	Screening and tracking severity in adults; item 9 screens for suicidal thoughts. A teen version (PHQ-A) also exists.	Self-report, 9 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
BDI-II (Beck Depression Inventory-II)	Measuring depression severity in teens and adults; widely used in research and practice.	Self-report, 21 items, 5 to 10 minutes	Yes	★ Level B	Pearson
BHS (Beck Hopelessness Scale)	Hopelessness, a strong warning sign for suicide; ages 17 to 80.	Self-report, 20 true/false items	Yes	★ Level B	Pearson
C-SSRS (Columbia Suicide Severity Rating Scale)	Screening and assessing suicide risk; brief screener and full versions.	Clinician- or staff-asked questions, a few minutes	No (free)	Free online training offered and recommended	Columbia Lighthouse Project
HAM-D (Hamilton Rating Scale for Depression)	Clinician rating of severity; the classic measure in treatment research.	Clinician interview, 21 items (17 scored)	No (free)	★ Skilled clinical interviewing; training recommended	Penn form (PDF)
GDS (Geriatric Depression Scale)	Older adults; avoids physical symptoms that can be due to aging or illness.	Self-report, yes/no; 15-item short or 30-item long form	No (public domain)	None beyond general clinical training	Stanford GDS page
EPDS (Edinburgh Postnatal Depression Scale)	Depression during pregnancy and after birth.	Self-report, 10 items, about 5 minutes	No (may be copied with credit to the authors)	None beyond general clinical training	NJ Health Dept. (PDF)
CDI 2 (Children's Depression Inventory, 2nd ed.)	Children and teens ages 7 to 17; includes parent and teacher forms.	Self-report, short and full forms	Yes	★ Level B	MHS
MMPI-3 (Minnesota Multiphasic Personality Inventory-3)	Broad personality and psychopathology assessment; places depression within the full clinical picture.	Self-report, 335 true/false items, about 25 to 50 minutes	Yes	★ Level C	Pearson
MCMI-IV (Millon Clinical Multiaxial Inventory-IV)	Adults in treatment; personality patterns alongside clinical syndromes such as depression.	Self-report, 195 true/false items, about 25 to 30 minutes	Yes	★ Level C	Pearson

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS uses similar levels. Check each publisher's page before ordering.

4. What works: psychological treatments with the strongest evidence

Bottom line for class: several structured psychotherapies work for adult depression with broadly similar effects. CBT has by far the largest evidence base. Psychotherapy is about as effective as antidepressants in the short term, and combining the two beats either one alone for moderate-to-severe depression.

Cognitive-behavioral therapy (CBT): the reference standard

- **Size of the evidence base.** A meta-analysis of **409 trials (52,702 patients)** found CBT had moderate-to-large effects against control conditions (g = 0.79). Its advantage over other psychotherapies was very small (g = 0.06) and was not robust. CBT matched antidepressants at the end of treatment and **outperformed them at 6–12-month follow-up** (g = 0.34). CBT combined with medication beat medication alone but did not beat CBT alone. CBT also worked as unguided self-help (g = 0.45) and in youth (g = 0.41) (Cuijpers et al., 2023).
- **Realistic expectations.** Across 228 trials of psychotherapy for depression, about **41% of patients responded** (50% symptom reduction), compared with about 17% under usual care. Only **about one-third remitted.** The NNT was about 5 versus usual care (Cuijpers et al., 2021). Teach students that many clients need more than one course of treatment, or a combination of treatments.

Behavioral activation (BA): the "behavioral core" of CBT

BA targets avoidance and withdrawal. Clients schedule valued, mastery, and pleasure activities, monitor how activity relates to mood, and solve problems around barriers.

- In a large RCT, **BA matched antidepressant medication among more severely depressed adults**, and both outperformed cognitive therapy in that subgroup (Dimidjian et al., 2006).
- A meta-analysis of 26 RCTs found BA superior to controls (SMD −0.74). Study quality was often low (Ekers et al., 2014).
- The **COBRA trial** (n = 440) found BA delivered by **junior mental health workers was non-inferior to CBT** delivered by professional therapists at 12 months, at lower cost (Richards et al., 2016). This makes BA a strong, teachable first-line option.

Cognitive therapy (CT) and relapse prevention

CT (Beckian) identifies and tests negative automatic thoughts and underlying beliefs, alongside behavioral work. For people with **recurrent depression who are in remission**, **mindfulness-based cognitive therapy (MBCT)** reduced the risk of relapse over 60 weeks compared with usual care (HR 0.69) and compared with active treatments, including antidepressants (HR 0.79) (Kuyken et al., 2016).

Other therapies with strong evidence

- **Interpersonal psychotherapy (IPT)** focuses on grief, role disputes, role transitions, and interpersonal deficits. It had moderate-to-large effects against controls (g = 0.60) and did not differ from other therapies or from pharmacotherapy. Maintenance IPT reduced relapse (Cuijpers et al., 2016).
- **Problem-solving therapy (PST)** is effective. Pooled effects were g = 0.79, but only **g = 0.34 in low-risk-of-bias trials**, which is comparable to other therapies. That gap is a useful teaching example of how bias inflates effect sizes (Cuijpers et al., 2018).
- **Combined treatment.** A network meta-analysis of 101 trials found psychotherapy plus medication more effective than either alone. Psychotherapy alone and medication alone did not differ, and psychotherapy was better tolerated (fewer dropouts) than medication (Cuijpers et al., 2020). Refer for a medication evaluation when depression is moderate-to-severe, chronic, psychotic, or not responding to treatment.

5. Christian and spiritually integrated interventions: what the evidence shows

Models. Religiously or spiritually integrated CBT keeps the standard CBT structure (thought records, behavioral activation, homework). It draws on the client's own faith resources: Scripture for challenging distorted thoughts, prayer and gratitude practices, service, forgiveness, and faith community as sources of activation and support. The best-known manualized protocol is **Religiously Integrated CBT (RCBT)** from Duke. It has versions for Christianity, Judaism, Islam, Buddhism, and Hinduism (Pearce et al., 2015).

Key findings:

- **Early RCT.** Among religious adults with depression, religious CBT and pastoral counseling outperformed standard CBT and a waitlist after treatment. Effects were equal across active treatments at 3-month and 2-year follow-up. The samples were small (about 10–20 per arm) (Propst et al., 1992).
- **RCBT vs. conventional CBT.** In 132 adults with MDD and chronic medical illness, the two produced **equivalent** outcomes overall. RCBT was slightly more effective, with somewhat better adherence, in **more religious** participants (Koenig et al., 2015).
- **Meta-analysis of faith-adapted CBT.** Sixteen RCTs for depression and anxiety showed statistically significant benefits. However, trials were **small and at high risk of bias**, with researcher and therapist allegiance a particular concern. The authors concluded that firm recommendations require larger rigorous trials (Anderson et al., 2015).
- **Broadest meta-analysis.** Across 97 studies, religiously or spiritually adapted therapy beat no treatment and modestly beat other secular therapies (psychological g = 0.33). In the **most rigorous additive designs**, which compare the same therapy with and without religious or spiritual content, results were **equivalent on psychological distress** (g = 0.13, not significant) but **better on spiritual well-being** (g = 0.34) (Captari et al., 2018).
- **Earlier reviews.** Several religious or spiritual therapies were judged helpful, with limited evidence that they outperform established secular therapies. The choice "may be an issue of client preference and therapist comfort" (Hook et al., 2010). A review of RCTs across mental health outcomes found the clearest religious/spiritual intervention benefits for anxiety and more modest, time-limited effects on depression (Gonçalves et al., 2015).

Honest summary for teaching:

1. Christian- or faith-integrated CBT is **about as effective as standard CBT** for reducing depression in religious clients. It is **not consistently superior**.

2. Its added value appears in **client fit, engagement, and adherence** and in **spiritual well-being**, especially for highly religious clients.
3. The evidence base is **smaller and weaker** than for standard CBT: fewer trials, small samples, allegiance effects, and few trials that are specifically Christian and of high quality.
4. **Practice ethics (general consensus):** integrate faith only when the client prefers it and after a spiritual assessment. Stay within your competence and role. Do not substitute integration for evidence-based structure or risk assessment. Watch for spiritual struggles such as guilt, feeling punished, or anger at God, which can maintain depression.

6. Quick classroom takeaways

- Diagnose on duration (≥2 weeks), count (≥5 of 9), core symptom (mood or anhedonia), and impairment. Always rule out bipolar disorder and assess suicide risk.
- Screen and track with a brief free tool such as the PHQ-9; use the C-SSRS for suicide risk. Fuller tests such as the MMPI-3 and MCMI-IV require advanced qualification.
- First-line psychotherapies are **CBT, BA, IPT, and PST**. Use MBCT or continued CBT for relapse prevention. Combine with medication for moderate-to-severe cases.
- **BA** is simple, scalable, and as effective as full CBT.
- **Faith-integrated CBT** is a legitimate, client-centered option for religious clients. It performs about as well as standard CBT, with added spiritual benefits, but the evidence is modest.

Sources note

All journal articles listed in References were checked against Crossref (Oct 28, 2026). Each DOI below is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites (Nov 1, 2026).

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